

PHARMACOVIGILANCE INSPECTION · 18 NOVEMBER 2026

The computerised system and database

Literature screening, the regulatory watch and the ICSR database of our pharmacovigilance, in one system.

THE NAME

Pharos was the lighthouse of Alexandria, the light that brought every ship safely into harbour. PHAROS keeps that watch for us: over the regulators of 25 reference countries, the medical literature and our own reports.

PRESENTED BY

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Qualified Person for Pharmacovigilance

SATO PHARMA

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Four parts, then the system itself

The computerised system and database of Sato Pharma's pharmacovigilance, presented by the QPPV.

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I

How we screen

Screening in two passes, vertical and horizontal, in every reference.

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One system for the screening we do every day

25

REFERENCE COUNTRIES

their regulators watched for us, with our local references added

15

LANGUAGES

each authority read as it publishes; nothing is translated

2

PASSES

by active ingredient, and by authority, so one covers what the other misses

0

RESULTS RECORDED TOO

a search that found nothing is kept: the evidence for negative results

BEFORE	WITH PHAROS
Each reference searched by hand, one ingredient at a time.	Every reference read on schedule, in its own language.
Positive results kept in an Excel sheet, with their PDFs.	Every result recorded, positive and negative, daily and weekly.
No record of a search that found nothing.	Each check kept with its date, its count and an archive copy.
ICSRs in an Excel log.	ICSRs in a database, sent as E2B or as the CIOMS I form.

Screening in two passes: vertical and horizontal

The way the screening was always done by hand, now done by the system.

	EMA	FDA	MHRA	BFARM	ANSM	PMDA	EDA	NCBI
Levofloxacin	■	■	■	■	■	■	■	■
Lornoxicam			■					
Guaifenesin			■					
Carvedilol			■					
Ivabradine			■					

■ Vertical: one ingredient, every reference

■ Horizontal: one authority, everything new

SHOWN: A FEW INGREDIENTS AND REFERENCES

VERTICAL PASS · INGREDIENTS TAB

We take each active ingredient of our products and search it in every reference: the regulators of the 25 reference countries, and NCBI on its own for the studies. PHAROS tells us whether any source, anywhere, mentioned it.

HORIZONTAL PASS · SOURCES TAB

We take each reference authority and look at everything it newly published: a round-up, a safety alert, a new PDF. We look by eye for anything that concerns us. This covers what the vertical search may miss.

Each active ingredient, in every language, with its group

Many class notices never name the ingredient: “fluoroquinolones” and not “levofloxacin”.

A DRAWING, NOT REAL DATA

ACTIVE INGREDIENT

READY

Levofloxacin

ATC CODE	J01MA12
PHARMACOTHERAPEUTIC GROUP	Fluoroquinolones
NAMES, 15 LANGUAGES	levofloxacin · lévofloxacine · Levofloxacin · levofloxacino · levofloxacina · levofloksacin · levofloksasiini · レボフロキサシン · 레보플록사신 · ليفوفلوكساسين

Every notice in any of these words, or in the group's words, is matched.

The group is searched with the ingredient

A notice about fluoroquinolones counts for levofloxacin, though it never names it.

Ready, or what is still being prepared

Its names, its group, the check of the past, the literature look-back and its labels.

Nothing screened is ever deleted

An ingredient taken off the list is paused, and its record stays.

Literature: global references and local references

Every search is logged with its date and its count, including a count of zero.

Global references

PubMed and PubMed Central

Through NCBI: both databases, not PubMed alone.

Europe PMC

The European literature database.

Cochrane

Counted through PubMed and Europe PMC by journal.

Medscape

Checked by hand, and the check recorded.

Local references

The Egyptian medical journals

Required by the EDA: Al-Azhar, Ain Shams and the others, each check recorded.

Egypt · the Egyptian Drug Authority

DHPC letters, safety alerts, awareness letters.

Saudi Arabia, Jordan, the United Arab Emirates

Kept, read in Arabic.

Added by the QPPV

A local reference is added from the Sources tab.

Every reference authority, everything newly published

Each authority is read in its own language: 15 languages, nothing translated.

European Union EMA EN	**Australia** TGA EN	**Austria** BASG DE	**Belgium** FAMHP FR NL	**Canada** HEALTH CANADA EN FR
Denmark DKMA DA	**Finland** FIMEA FI SV	**France** ANSM FR	**Germany** PEI · BFARM DE	**Iceland** IMA IS
Ireland HPRA EN	**Italy** AIFA IT	**Japan** PMDA JA	**Luxembourg** DPM FR	**Netherlands** CBG-MEB NL
New Zealand MEDSAFE EN	**Norway** DMP NB	**Portugal** INFARMED PT	**Singapore** HSA EN	**South Korea** MFDS KO
Spain AEMPS ES	**Sweden** MPA SV	**Switzerland** SWISSMEDIC DE FR IT	**United Kingdom** MHRA EN	**United States** FDA EN

LOCAL AUTHORITIES

Egypt	EDA
Jordan	JFDA
Saudi Arabia	SFDA
United Arab Emirates	MOHAP · EDE

INTERNATIONAL

World Health Organization · medical product alerts

The QPPV adds, changes or removes a link herself. Where a site cannot be read automatically, PHAROS says so, gives the reason, and the page is checked by hand.

II

The evidence

Evidence that we searched, including when the result was negative.

Evidence for negative results 10

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Reports 12

It searched, and it found zero: that is recorded

Positive results were always documented. The evidence that a search found nothing is now kept too.

A DRAWING, NOT REAL DATA

WEEKLY REPORT · PAGE 3 OF 12

Screening log · week of Sunday 8 November 2026

ACTIVE INGREDIENT	AUTHORITY	WHAT WAS READ	READ	RESULT
Levofloxacin	MHRA · United Kingdom	Drug Safety Update, alerts	7 of 7 days	**Nothing found**
Levofloxacin	PMDA · Japan	Safety notices, Japanese	7 of 7 days	**Nothing found**
Levofloxacin	NCBI · PubMed, PMC	Search, with its count	7 of 7 days	**0 new papers**
Lornoxicam	ANSM · France	Safety notices, French	7 of 7 days	**1 notice: not ours**
Guaifenesin	FDA · United States	Drug Safety Communications	7 of 7 days	**Nothing found**
Carvedilol	EDA · Egypt	DHPC letters, safety alerts	7 of 7 days	**Nothing found**

SHA-256 9C41 7E0B 2A55 … 52D1

SIGNED · REVIEWED BY THE QPPV

Daily and weekly

For every active ingredient on its own, and across all of them.

Weekly, never monthly

Some findings must reach the authority within five days.

Kept, unchangeable

Every page read has an archive copy that nothing can overwrite or delete.

From a finding to the authority, within five days

The clock counts from the day PHAROS first showed the notice.

01

A new notice

Matched to the ingredient, its group or one of its names.

02

Our decision

Concerns us, or doesn't concern us, with who decided and when.

03

The five-day clock

Due at the Egyptian Drug Authority on day five; late is shown in red.

04

Sent to the authority

The date sent and the reference are recorded; the clock stops.

THE CLOCK, DRAWN TO SCALE



Day 0: the day PHAROS first showed the notice

Sent on day 3: clock stopped

Day 5: due

Reports the inspector can hold

Every report is a PDF with its Excel workbook, made by PHAROS from the same records.

EVERY SUNDAY

Weekly report

- Every active ingredient, every authority
- Every zero result, with what was read
- Decisions and what went to the authority
- Possible case reports with their due dates

ON REQUEST, ANY INTERVAL

PSUR pack

- Regulatory actions by country
- Label changes, compared with our label
- Literature, case reports first
- The screening log of the interval

ON REQUEST

Inspection pack

- Coverage: every page and how it is read
- The screening log, decisions, sign-offs
- Reports to the authority
- Quality: deviations, audits, CAPA plan

Each file carries its SHA-256 fingerprint and can be signed as Reviewed or Approved. The history of every record is kept, and nobody can change it.

III

Cases and quality

The ICSR database, and one CAPA plan for deviations and audits.

The ICSR database 14

Quality 15

The ICSR database

ICSRs were kept in an Excel log, an approved alternative while our numbers are low. Now every ICSR is in a database.

01 **New case**
Opened from the Cases tab, numbered, dated.

02 **The CIOMS form**
Dropped into the case; PHAROS reads it into the case's fields.

03 **Our product**
From the product list, with its strength: carvedilol with ivabradine.

04 **MedDRA coding**
From the company's MedDRA subscription, once its files are loaded.

05 **Sent**
As an E2B (R3) file; as the CIOMS I form until MedDRA arrives.

A DRAWING, NOT REAL DATA

CASE **2026-0007** — OPEN · NON-SERIOUS

RECEIVED	Monday 9 November 2026 · CIOMS form, PDF
PRODUCT	Carvedilol 6.25 mg with ivabradine 5 mg
REACTION	Coded to a MedDRA preferred term
REPORTER	Health care professional, Egypt
DUE	Day 0: 9 Nov · 90 days: Sunday 7 February 2027
SEND AS	**E2B (R3) file** **CIOMS I form**

A CAPA comes from a deviation, or from an audit

Both end in one CAPA plan. PV-SOP-024, the Quality Management System, is the overview of both.

PV-SOP-021

Deviation

A mistake happened. Level 1, 2 or 3; investigated; its root cause found; approved by the QA manager.

PV-SOP-022

Audit observation

Internal, every last quarter, or external. Critical, major or minor; deadlines from day 1 to day 14.

ONE PLAN

The CAPA plan

Corrective and preventive action, owner, due date, evidence.

UNTIL

Effective

Not effective sends it back. A deviation closes when every CAPA of it is effective.

The 07:00 morning message lists the CAPAs and audit deadlines that are due or late. Nothing is deleted; every step is kept in a history nobody can change.

IV

Control

Who can do what, what can never change, and where our SOP stands.

People and integrity 17

SOP and master file 18

Who can do what, and what can never change

People and roles

- Each person signs in with their own account
- Readers see everything and change nothing
- Reviewers decide; administrators add people and sources
- Linked people get the morning message on Telegram

Records that stay

- Every page read is archived by its fingerprint
- The archive cannot be overwritten or deleted
- Every decision and change kept with who and when
- An encrypted copy of the database every night

Where it opens

- In the browser, on the laptop and the phone
- Installs as an application on the taskbar
- Every time shown in Cairo time
- The handbook describes every screen

In use now; our SOP and master file follow

The process is new, so it is not yet in the master file or an SOP. Until the new SOP is approved, the current one stands.

5 NOV 2026

Defined

The QPPV sets the two passes: vertical by ingredient, horizontal by authority.

8 NOV 2026

Adopted

For literature screening. The Excel sheets retired; the ICSRs move to the database.

9 NOV 2026

Quality

Deviations, audits and one CAPA plan, from PV-SOP-021, 022 and 024.

18 NOV 2026

Today

Presented as the computerised system and database.

NEXT

Written in

The literature-screening SOP and the PV master file describe PHAROS.

The PHAROS handbook describes each screen in words that can be lifted into the SOP and the master file.

PHAROS

Pharmaceutical Alerts & Regulatory Observation System

NOW, ON THE SYSTEM ITSELF

Live demonstration

01	Ingredients	the vertical pass
02	Sources	the horizontal pass
03	Reports	the weekly report, the packs
04	Cases	the ICSR database
05	Quality	deviations, audits, CAPA

Dr Sama Mosaad · QPPV

SATO PHARMA



Thank you

QUESTIONS ARE WELCOME

Dr Sama Mosaad · Qualified Person for Pharmacovigilance

SATO PHARMA · PHARMACOVIGILANCE INSPECTION · WEDNESDAY 18 NOVEMBER 2026